

Guide 01 — Community Health Worker

Controlled English working master — version 1.1-draft

Review date: August 5, 2026 **Audience:** Adults exploring entry, transition, or advancement pathways in community health work

Status: Substantive English working master completed from the controlled official-source baseline. Legacy comparison, trilingual integration, sentence-level editorial review, structural-parity and safety review, terminology normalization, market-income reconciliation, and controlled link review are recorded. The trilingual editorial and terminology freeze is complete. Final live-link and dated market-estimate validation, DOCX/PDF generation, metadata, checksums, rendering, and publication QA remain pending.

This guide provides general career and education information. Requirements differ by country, state, province, territory, employer, funding program, and job title. It does not provide medical, legal, immigration, licensing, accreditation, or financial advice. Completion of a course, certificate, internship, apprenticeship, or volunteer placement does not guarantee employment, certification, registration, licensure, promotion, or income.

1. What a community health worker does

A community health worker, often shortened to **CHW**, helps people connect with health care, public-health programs, social services, and practical community resources. The role is built on trust, cultural understanding, communication, and knowledge of the community served.

Common titles include:

- community health worker;
- promotora or promotor de salud;
- peer navigator;
- patient navigator;
- outreach worker;
- community health representative;
- lay health adviser;
- health coach;
- family or community support worker.

Titles are not uniform. Two employers may use the same title for different duties, and two workers may perform similar duties under different titles. Always read the job description and verify local requirements.

Typical responsibilities may include:

- explaining health and social-service information in plain language;
- helping people schedule appointments and understand referrals;
- connecting individuals and families with transportation, food, housing, benefits, or language support;
- supporting prevention, screening, chronic-disease education, and follow-up under an approved program;
- conducting outreach in homes, schools, clinics, faith communities, shelters, workplaces, or community events;
- documenting services and outcomes;
- identifying barriers related to social determinants of health;
- building trust between communities and health or social-service organizations;
- escalating urgent, clinical, safeguarding, or privacy concerns to qualified personnel.

A CHW is not automatically a nurse, physician, psychologist, social worker, therapist, or other licensed clinician. CHWs must remain within the duties authorized by their employer, program, training, supervision, and local law.

2. Who may be a good fit

Community health work may suit people who are patient, dependable, culturally respectful, and comfortable helping others navigate complex systems. Lived experience, community credibility, bilingual ability, and knowledge of local resources can be major strengths.

Useful qualities include:

- active listening and empathy;
- clear spoken and written communication;
- respect for privacy and informed choice;
- reliability and accurate recordkeeping;
- conflict de-escalation;
- cultural humility;
- basic digital skills;
- willingness to work with multidisciplinary teams;
- ability to maintain professional boundaries;
- readiness to travel locally or work outside a traditional office.

The work can be emotionally demanding. Employers should provide supervision, safety procedures, escalation routes, and support for difficult cases.

3. Education, training, and credentials

There is no single worldwide CHW qualification. Requirements vary by jurisdiction and employer.

United States

The U.S. Bureau of Labor Statistics identifies a high school diploma or equivalent as the typical entry-level education and short-term on-the-job training as typical. This national profile does not override state certification rules or employer requirements.

Some states have formal CHW certification, approved training programs, competency standards, registries, or Medicaid-related requirements. Other states rely more heavily on employer training or voluntary credentials. Before enrolling, check:

1. whether the program is recognized by the state or employer you are targeting;
2. whether supervised field experience is required;
3. whether background checks, immunizations, driving, language, or technology requirements apply;
4. whether the credential is voluntary, employer-specific, state-recognized, or required for reimbursement.

Canada

The Government of Canada Job Bank maps the title “community health worker” to **NOC 42201, Social and community service workers**, a broader occupational group than the U.S. CHW category. Job Bank reports that a college or university program in social work, child and youth care, psychology, or another social-science or health-related field is usually required, although relevant volunteer or support experience may replace formal education in some roles.

Requirements differ by province, territory, health authority, Indigenous organization, and employer. Community health nursing is a separate regulated occupation and should not be confused with CHW or community-support roles.

Colombia

Possible pathways include work with:

- Empresas Promotoras de Salud (EPS);
- Instituciones Prestadoras de Servicios de Salud (IPS);
- Empresas Sociales del Estado (ESE);
- municipal or departmental health secretariats;

- universities and public-health projects;
- nongovernmental organizations;
- community-based organizations;
- humanitarian or social-development programs.

Colombia's integrated health pathways and social-participation policies include community and intersectoral action. Requirements depend on the employer, contract, program, territorial authority, and assigned duties. Authorization of a community-based organization does not automatically create an individual professional license for every worker.

SENA, territorial health authorities, universities, employers, and recognized community organizations are appropriate places to check for current training and work-based pathways.

Other Latin American countries

Community health roles may appear within ministries of health, municipal programs, primary-care networks, NGOs, universities, humanitarian organizations, or donor-funded projects. Titles such as *promotor de salud*, *agente comunitario de salud*, *facilitador comunitario*, and *navegador de pacientes* may not be equivalent across countries.

Verify:

- the legal and institutional status of the role;
- whether the work is paid employment, contract work, volunteer service, or a stipend-supported project;
- whether a health, social-work, technical, or community-development credential is required;
- whether local-language, Indigenous-language, driving, residency, or fieldwork requirements apply.

4. Free and low-cost learning first

Before paying for a private program, use free official resources to confirm that the occupation fits your goals.

United States resources

- **CDC Resources for Community Health Workers:** role information, tools, and public-health resources.
<https://www.cdc.gov/chronic-disease/php/community-health-worker-resources/index.html>
- **CDC cardiovascular training resource:** free material for CHWs with some experience; it is not a replacement for basic CHW training or a universal credential.
https://archive.cdc.gov/www_cdc_gov/dhdsp/programs/spha/chw_training/index.htm

- **American Job Centers:** local workforce counseling, training eligibility, and employer connections.
<https://www.careeronestop.org/LocalHelp/AmericanJobCenters/american-job-centers.aspx>
- State health departments, state CHW associations, community colleges, federally qualified health centers, and public-health departments may publish approved or employer-linked programs.

Canada resources

Use:

- provincial or territorial employment services;
- public colleges and universities;
- health authorities;
- Indigenous health organizations;
- settlement and community-service organizations;
- employer-sponsored training.

Start with the Government of Canada Job Bank occupation pages and then verify regional requirements.

Colombia resources

Check:

- SENA course and employment services;
- Servicio Público de Empleo;
- territorial health secretariats;
- EPS, IPS, and ESE vacancy and training pages;
- public universities;
- recognized NGOs and community-based organizations.

Do not assume that every short online course is recognized by employers or authorities.

5. Funding, scholarships, and employer support

Funding is eligibility-dependent and changes frequently. Ask for the written terms before enrolling.

Possible support includes:

- public workforce training funds;
- community-college grants or fee waivers;
- employer-paid onboarding or certification;
- tuition reimbursement;

- scholarships from health systems, foundations, unions, or community organizations;
- paid internships, traineeships, apprenticeships, or service placements;
- transportation, childcare, equipment, or examination assistance;
- stipends for project participation.

Keep the categories separate:

- **wage:** pay for employment;
- **stipend:** fixed support that may not create employee status;
- **scholarship or grant:** education funding that may have conditions;
- **loan:** money that must normally be repaid;
- **reimbursement:** payment after an approved expense;
- **apprenticeship:** structured paid work and related instruction when formally established;
- **certificate of completion:** evidence of completing a course, not automatically a professional license or academic award.

Never pay a recruiter for a job offer. Be cautious of programs that guarantee employment, certification, immigration results, or unusually high income.

6. Apprenticeships and paid work-based learning

Formal CHW apprenticeships exist in some U.S. locations, but availability is regional. Search the official Apprenticeship.gov finder and verify the sponsor, occupation, location, pay, eligibility, and related instruction.

<https://www.apprenticeship.gov/apprenticeship-job-finder>

Other legitimate work-based options may include:

- employer-paid trainee roles;
- public-health internships;
- community-service placements;
- supervised practicum experience;
- union or health-system career pathways;
- paid research or outreach projects.

In Canada and Latin America, the term “apprenticeship” may not be used for CHW pathways. Look for supervised placements, traineeships, practicums, employer training, or paid community-program roles, and verify whether the arrangement creates employee, student, contractor, volunteer, or stipend status.

7. Income and employment outlook

Income figures are reference points, not promises.

United States official data

The U.S. Bureau of Labor Statistics Occupational Outlook Handbook, last modified August 28, 2025, reports for Community Health Workers:

- median annual wage in May 2024: **USD 51,030**;
- median hourly wage: **USD 24.54**;
- employment in 2024: **65,100 jobs**;
- projected growth from 2024 to 2034: **11%**, much faster than average;
- approximately **7,800 openings per year** on average in the narrative projection.

Source: <https://www.bls.gov/ooh/community-and-social-service/community-health-workers.htm>

Pay varies by state, metropolitan area, employer, schedule, union coverage, duties, language skills, experience, and certification requirements. The median is not an entry wage.

United States supplementary market estimates

Current non-government estimates provide additional context but are not official statistics and must not replace the BLS figures above. Glassdoor displayed an estimated **USD 52,000 median total pay** for Community Health Workers in the United States, with a typical total-pay range of approximately **USD 44,000 to USD 63,000**, when revalidated on August 6, 2026. ZipRecruiter reported an estimated average annual pay of **USD 44,925** as of July 27, 2026, using employer job postings and third-party data.

These estimates use different methods, samples, and update cycles. Do not average them together, treat them as guaranteed offers, or assume that they apply to a particular state, employer, experience level, or employment arrangement.

Canada official data

The Government of Canada Job Bank national wage table for NOC 42201, updated November 19, 2025 and displayed on a page modified June 2, 2026, reports:

- low: **CAD 19.00 per hour**;
- median: **CAD 26.00 per hour**;
- high: **CAD 36.06 per hour**;
- **83.6%** receive at least one non-wage benefit.

Source:

<https://www.jobbank.gc.ca/marketreport/wages-occupation/296075/ca>

These figures cover the broader NOC 42201 group. They are not guaranteed CHW offers. Review the province, territory, and region that applies to you.

Colombia and Latin America

No single Colombian wage series was verified in this batch as directly equivalent to the U.S. BLS occupation. Do not convert U.S. or Canadian wages into Colombian salary expectations.

For a local estimate, identify the exact employer, location, contract type, schedule, duties, and required education. Give greater weight to:

1. official public-sector salary schedules;
2. disclosed employer vacancies;
3. recognized labor-market sources;
4. clearly labeled market estimates with geography and sample limitations.

Keep salaries separate from volunteer service, stipends, project payments, grants, and reimbursements.

8. Building experience ethically

A beginning portfolio can show communication, navigation, and community-resource skills without exposing private information.

Safe examples include:

- a public-resource directory using official links;
- a plain-language health-navigation handout reviewed against an official source;
- a mock outreach plan using fictional people and organizations;
- a bilingual glossary of non-clinical navigation terms;
- a referral workflow diagram with no real patient data;
- a reflection on cultural humility, boundaries, and escalation.

Do not include names, medical histories, addresses, photographs, identification numbers, appointment details, case notes, or other identifiable information from real people.

9. Responsible use of artificial intelligence

AI can help organize public information, improve readability, translate a draft for later review, or generate practice scenarios. It must not replace professional judgment, local policy, clinical advice, privacy controls, or qualified translation where accuracy is critical.

Use these safeguards:

- never enter patient, client, applicant, employee, or community-member personal information into an unapproved AI service;
- verify health and benefits information against current official sources;
- disclose AI assistance when required by an employer, school, funder, or publication policy;
- have qualified reviewers check translated or clinically sensitive material;
- do not use AI to diagnose, prescribe, determine eligibility, or make emergency decisions;
- follow employer-approved systems and retention rules.

10. Job-search checklist

Before applying:

1. Confirm the exact title and duties.
2. Check whether certification, registration, education, driving, background screening, immunization, language, or travel is required.
3. Verify whether the position is employee, contractor, temporary, volunteer, internship, apprenticeship, or stipend-based.
4. Compare the pay period correctly: hourly, monthly, annual, per visit, or per project.
5. Ask about supervision, safety, caseload, travel reimbursement, technology, documentation, and working hours.
6. Tailor the résumé honestly around community knowledge, navigation, outreach, language skills, documentation, and trust-building.
7. Never claim clinical authority or credentials you do not hold.

11. First 30-day action plan

Week 1 — Explore

- Read the CDC and BLS occupation pages or the official sources for your country.
- Review ten current local job descriptions.
- List repeated requirements and titles.

Week 2 — Build foundations

- Complete one free official learning resource.
- Create a local resource directory.
- Practise plain-language explanation and referral documentation.

Week 3 — Verify pathways

- Contact a workforce center, public college, SENA office, health authority, CHW association, or employer.
- Ask which programs employers actually recognize.
- Compare funding, field experience, schedule, and total cost.

Week 4 — Apply carefully

- Prepare a truthful résumé and short cover message.
- Apply to realistic trainee, outreach, navigator, or CHW positions.
- Track applications, follow-ups, requirements, and outcomes.

12. Controlled source register

- Centers for Disease Control and Prevention, Resources for Community Health Workers, page dated December 3, 2024:
<https://www.cdc.gov/chronic-disease/php/community-health-worker-resources/index.html>
- CDC archived cardiovascular CHW training resource, reviewed April 27, 2022:
https://archive.cdc.gov/www_cdc_gov/dhdsp/programs/spha/chw_training/index.htm
- U.S. Bureau of Labor Statistics, Occupational Outlook Handbook, Community Health Workers, modified August 28, 2025:
<https://www.bls.gov/ooh/community-and-social-service/community-health-workers.htm>
- Glassdoor, Community Health Worker salary estimate for the United States, accessed August 6, 2026; estimated median total pay USD 52,000 with a typical total-pay range of approximately USD 44,000–63,000: https://www.glassdoor.com/Salaries/community-health-worker-salary-SRCH_KO0,23.htm
- ZipRecruiter, Community Health Worker salary estimate for the United States, dated July 27, 2026; estimated average USD 44,925 using employer postings and third-party data:
<https://www.ziprecruiter.com/Salaries/Community-Health-Worker-Salary>
- Government of Canada Job Bank, Community Health Worker summary and requirements, modified June 2, 2026:
<https://www.jobbank.gc.ca/marketreport/summary-occupation/296075/ca> and
<https://www.jobbank.gc.ca/marketreport/requirements/296075/ca>

- Government of Canada Job Bank wage report, modified June 2, 2026:
<https://www.jobbank.gc.ca/marketreport/wages-occupation/296075/ca>
- Ministerio de Salud y Protección Social, Rutas Integrales de Atención en Salud:
<https://www.minsalud.gov.co/salud/publica/ssr/Paginas/Rutas-integrales-de-atencion-en-salud-RIAS.aspx>
- Ministerio de Salud y Protección Social, Política de Participación Social en Salud, policy foundation Resolution 2063 of 2017:
<https://minsalud.gov.co/Participa/Paginas/Consulta-ciudadana-derecho-a-la-informacion-publica.aspx>

13. QA and review status

Completed for this working master:

- official-source integration;
- occupation-scope controls;
- U.S., Canada, Latin America, and Colombia pathways;
- funding and free-first learning guidance;
- apprenticeship and work-based-learning controls;
- controlled U.S. and Canadian income figures;
- privacy and responsible-AI safeguards;
- plain-language structure and accessibility-oriented headings;
- explicit limitations on certification and employment claims.

Still required before publication:

- validate every external link from the final candidate environment;
- revalidate the supplementary non-government market estimates immediately before publication and retain their methodology limitations;
- generate DOCX and searchable PDF candidates;
- verify OOXML, hyperlinks, PDF text, metadata, page rendering, filenames, checksums, and publication manifests;
- do not describe automated review as independent human certification, professional translation certification, accreditation review, accessibility certification, medical review, or legal review.